

“It destroys families, communities and futures”: local knowledge to reduce alcohol and drug abuse

Alcohol and other drug (AOD) abuse I nkingha leyi kulu leyi yi nga kona laha South Africa. Ku dya swidzidziharisi leswi swina swita ndzhaku leswi kulu ngopfu, leswi swi fambelanaka na mahanyelo lawa ya katsaka timhaka ta masangu, vugevenga, madzolonga, na swilo hi nkwaswo leswi swi katsaka rihanyo, care seeking, ku tirhisa na ku tiyisela loko swita ka treatment, ti nkingha leti ti katsaka ku hangalaka ka mavabyi ya HIV, transmission, and conditions including heart disease, cancers, stroke and seizures. Ku tirhisana na matiko ya local, nkongomelo wa ndzavisiso lowu iku twisisa na ku lava ku lulamisa xiphiko lexi xa AOD abuse in Mpumalanga.



Increasing numbers of taverns reported

Byala na swi dzidzharisi swi n’wana swina switandzhaku swo tala loko swi tala eka rihanyo ra vanhu, e tikweni/ ka vanhu hiku angarhela na economy ya tiko. Xiphiko lexi xa lulamiseka na swona swita ndzhaku swa xiphiko lexi swi vavisa ngopfu lava va nga swisiwana/va tshamaka matiko xikaya. Ndzavisiso lowu wu endlwaka hiti nomboro wa vanhu lava va nga lova lowu wu endlwaka hi MRC/wits Agincourt ku katsa na ndzavisiso lowu wu endlwaka e tikweni swi langutisiwile, laha aku tirhiwa na vanhu lava kulu, na vantshwa. Ku kumiwe na vumbhoni lebyi seketelaka vuxokoxoko lebyi byi nga kumiwa bya tinomboro.

*“Yesterday I saw blood flowing like water at the tavern”
Community participant*

Drivers of AOD abuse

- **Taverns:** kuna swipoti leswi nyingi leswi swi pfalaka navusiku kumbe matakuxa, swi n’wana swa swona aswi pfali. Ka ti ndhawu leti ka gembuliwa na swona ku thyakile swi poti kumbe ti tarven ti ripotiwele tani hi swilo leswi swi endlaka leswaku ku va naku hangalaka ka swidzidziharisi, ku endlwaka ka masangu na vanhu va nga tisirhelelangi, ku xavisa miri, na ku hlukumezana kumbe ku teka swidzidziharisi swo fananati drugs. Vana va sungula ku dzaha no n’wa byala hikokwalaho kova swi nga tiki kuva swi kumeka eti tarven. *ku tlakuka ka ti nomboro tati tarven leti ripotiweke*
- **Poverty:** swilo swo hlaya leswi vangaka/swi vangelo swo hlaya nyana: vusiwana, ku pfumala ka dyondzo na kuva mintirho yi ngari kona. Vana va n’wana va tsandzeka ku amukela leswaku vasiwanekile ku tani swi va endla leswaku va teka swi dzidziharisi ku susa xitirese.
- **Modern culture:** vanhu lava kulu va kombise leswaku swi vangelo swi n’wana i TV, media and peer pressure, va kombise leswaku mahanyelo ya masiku lawa ama fambelani na ndhavuko wa Xichanganga hi kuva ndhavuko wa hina wu vula leswaku munhu i fanele ati xixima a tlhela a xixima va n’wana. Kambe vantshwa va nyike mavonelo yo hambana, va vula leswaku kuva munhu anga tirhisi swi dzidziharisi aswimu endli ava munhu wa le hansi.
- **Substance abuse:** vanhu va tirhisa swilo swo fana na marijuana, benzene and glue. Ku ripotiwe leswaku vana lava ntsongo vateka ma ARVs va ya hlanganisa na swilo ya endla Nyaupe ku tani vaswi dzaha.
- **Traditional practices:** Several reported e.g. traditional beer (Xipayoni).
- **Corruption:** vanhu lava va xavisaka swidzidziharisi va hlangana na maphorisa na va rhangeri va le tikweni ku endla swilo leswi
- **Adults/elder distress and anxiety:** kuva na ntshikelelo wo karhi loko munhu a tirhisa swi dzidziharisi, e.g. kuva naku yiva emuntini, va hlukumeza va n’wana va maxaka hi swa masangu.
- **Lack of planning and leadership:** e mi ndyangwini, e tikweni, eka varhangeri, na ku pfumala ka vuxokoxoko byo pfuna vantshwa.

*“People selling alcohol serve no purpose... If we had power, we wouldn’t have taverns in this community”
Community participant*

Impacts of AOD

- **Behavioural:** many reported: vudyoho ekaku seketela ku vavanhu va xanisiwa hi swidzidziharisi, ku gembula na ku teka swinqumo swo ka swi ngari kahle, ku chayela uri karhi u dakwile, ku endla ti mhaka ta masangu u ngati sirhelelanga, ku xavisa miri, ku lahlekeriwa hi xinzuti, kuka munhu anga landzelerisi mirhi leyi va n'wi nyikeke yona, mahanyelo yoka yangari kahle yo fana no rhandza tinyimpi, ti ngozi, kuka u nga dyi kahle, ku pfumala ku ti hlayisa na ku yiva.
- **Health:** many reported: ku hlukumeziwa (leswi swi katsaka na ku hlukumeziwa hiti mhaka ta masangu), cancer, disability, HIV/AIDS, injuries (including RTAs), malnutrition, mental health, overdose, stress, stroke, suicide, TB, heart, lung, liver and other vital organ conditions and unplanned pregnancy.
- **Social:** swita ndzhaku leswi fambelanaka na dyondzo, ku pfumala ka mintirho na na vuswi koti byo hanya vutomi bya kahle, vugevenga na jele.
- **Destroys future, destroys communities:** hiku langutisisa hinkwaswu leswi swi endliwaka hi swi dzidziharisi hi nga vula leswaku swi dzidziharisi swi onha miminti na tiko hiku angarhela.

Deaths linked to drug/alcohol abuse (Agincourt HDSS 2014-15)

Road traffic accident	42	***
Self-harm/suicide	22	***
Assault	21	***
HIV-related	185	**
Lung cancer	36	**
Liver damage	24	**
Other transport accident	17	**
Other injury	4	**
TB	104	*
Stomach cancer	70	*
Heart attack	47	*
Heart disease	44	*
TOTAL	513	**

* May be related to substance abuse
 ** Likely to be linked to substance abuse
 *** Very likely to be linked to substance abuse

Statistical data 2014-15

- Of 1,524 deaths >500 may have been due to/influenced by, AOD abuse.

Actions recommended

Reduce rates of AOD abuse: Communities developed integrated action plans, inclusive of multi-sectoral partnerships and monitoring mechanisms to achieve realistic and achievable reductions in rates of AOD abuse as follows [swilo leswi swi nga pulaniwa leswaku swi lava ku endlewa leswaku swi lo leswi swi ta lulamisiwa]:

- **Regulate taverns:** ku ehliwa minkarhi/ti awara leyi ti tarven ti pfulaka yona, ku ehliwa nhlayo ya ti tarven, ku endla leswaku loko swita eka malembe ya lava va ngenaka ti tarven swi tikisiwa, ku nga endliwi swilo leswi kokaka mahlo ya vantshwa leswi swi ngata va endla leswaku va tikuma vava ti tarven (e.g. pool tables), maphorisa swi lava ma kumeka ngopfu eti tarven, ti tarven ti fanele ku tshama ti kamberwa, na tiko hinkwaru ri fanele ku tshama ri langutisisa ri tlhela ri kamberwa, ku katsa na community police forum (CPF) swi lava va ngenenelela eka ku kambisisa ti tarven, Indunas swi lava ti tirha nava rhangeri vati tarven leswaku vata lulamisa vuxaka exikarhi ka vona na tiko.
- **Education and employment:** Registers leti ti ngata kombisa leswaku kuna ku landzelerisa, ku fanele ku kamberwa vana eswi kolweni ku tlhela ku endliwa ti test, va dyondzisi na vatswari va fanele ku hlangana va pfunana ku kurisa vana. Ku hlukukisa swilo leswi vana va faneleke ku swi endla loko va huma ka metiriki (e.g. bursaries). Ku endla leswaku ku va na mintirho via community education, rehabilitation and recreation, local garden farming initiatives etc. (see below). In other settings, NGOs and social enterprises address social needs via service provision delivered through “supported employment and volunteering” to break cycles of addiction and poverty.
- **Community rehab and drop-in:** improved referral to existing support services, and building of new rehab facilities, youth drop-in recreation centres with authorities (e.g. Depts. of Health, Education and Social Development, National Council SANCA) and NGOs.
- **Community health education:** ku dyondzisa vanhu mayelana naku teka swi dzidziharisi eti tliliniki na le swikolweni. Vantshwa vanghenelela ka swi dzidziharisi hiku pfumala nseketelo na ku seketeriwa hi swati mali. More teachers, social workers, emotional support, food aid and housing support for people living in poverty.
- **Community partnerships:** ti nduna leti ti ngana matimba e tikweni ti fanele ku tirhisana na ti ward committees, parents, CPF, community development forum (CDF), pastors, magistrates, community health workers, schools. Councillors to work effectively for community.
- **Research:** kuya emahlweni naku tirhisana na wits ku twisisa kutikeriwa loko swita eka rifu leri rivangiwaka swidzidzihari.

Visual evidence



Drunk driving in youth



Traditional beer



Marijuana use

Verbal Autopsy with Participatory Action Research (VAPAR) is a programme of partnerships research funded the Health Systems Research Initiative from Department for International Development (DFID)/Medical Research Council (MRC)/Wellcome Trust/Economic and Social Research Council (ESRC) (MR/N005597/1) and MR/P014844/1). Image credits: © VAPAR 2017. Permissions have been secured for the reproduction of images.